

HOVID ACEPRIN-150 TABLET

Description

Light brown to brown color enteric coated round and biconvex tablets plain on both sides.

Composition

Each tablet contains:
Aspirin 150 mg.

Actions and Pharmacology

The platelet aggregation inhibiting effect of aspirin specifically involves the compound's ability to act as an acetyl donor to the platelet membrane. Aspirin inhibits the enzyme prostaglandin cyclooxygenase in platelets, thereby preventing the formation of the aggregating agent thromboxane A_2 . This action is irreversible; the effects persist for the life of the platelet exposed. Aspirin may also inhibit formation of the platelet aggregation inhibitor prostacyclin (prostaglandin I_2) in blood vessels; however, this action is reversible and may be dose-dependent.

Pharmacokinetics

Aspirin is absorbed rapidly from the gastrointestinal tract after oral administration. After absorption, aspirin is rapidly converted to salicylate but during the first 20 minutes following oral administration, aspirin is the predominant form of the drug in the plasma. Aspirin is 80 to 90% bound to plasma proteins and is widely distributed; its volume of distribution is reported to be 170ml per kg body weight in adults. Salicylate is mainly eliminated by hepatic metabolism; the metabolites include salicylic acid, salicyl phenolic glucuronide, salicylic acyl glucuronide, gentisic acid and gentisuric acid. Salicylate is also excreted unchanged in the urine, the amount excreted by this route increases with increasing dose and also depends on urinary pH, about 30% of a dose being excreted in alkaline urine compared with 2% of a dose in acidic urine.

Indications

As a platelet aggregation inhibitor in the prophylaxis of myocardial infarction, transient ischemic attacks or stroke, thromboembolism and vascular occlusion.

Contraindications

Except under special circumstances, this medication should not be used when the following medical problems exist:

- bleeding ulcers
- hemorrhagic states
- hemophilia or other bleeding problems, including coagulation or platelet function disorders
- angioedema
- nasal polyps associated with asthma
- thrombocytopenia
- hypersensitivity to aspirin

Precautions

Aspirin should be cautiously employed in patients prone to dyspepsia or known to have lesion of the gastric mucosa. It should not be administered to patients with haemophilia or other haemorrhagic disorders or gout. Aspirin should be used with caution in patients with asthma or allergic disorders. It should not be given to patients with a history of sensitivity reactions to aspirin or other NSAIDs. Aspirin should be avoided in severe renal or hepatic impairment. Aspirin should not be given to children under 16 years of age because of the risk of Reye's syndrome.

Use in pregnancy and lactation

Salicylate readily crosses the placenta and is distributed into breast milk. It has been reported that salicylate use during pregnancy may increase the risk of birth defects in humans. High dose of salicylate in late pregnancy may result in prolonged labor, complicated deliveries, and increased risk of maternal or fetal hemorrhage. It is advised not to take aspirin during the last trimester of pregnancy unless the therapy is prescribed and monitored by physician.

Main Side/Adverse Effects

The most common adverse effects occurring with therapeutic doses of aspirin are gastrointestinal disturbances such as nausea, dyspepsia and vomiting.

Drug Interactions

Concurrent use of aspirin and dipyridamole may result in an increase peak plasma-salicylate concentration and area under the curve. Serum-salicylate concentrations may be reduced by concurrent administration of corticosteroids. Aspirin may enhance the activity of coumarin anticoagulant, sulphonylurea, hypoglycaemic agent, methotrexate, phenytoin and valproic acid and inhibit the action of uricosuric.

Overdose

Tinnitus and/or headache may be the earliest symptoms of salicylism. Bloody urine, convulsions, hallucinations, severe nervousness, excitement or confusion, shortness of breath or troubled breathing, unexplained fever are the symptoms of severe overdose.

Treatment of overdose:

- Emptying the stomach via induction of emesis or gastric lavage
- Administering activated charcoal
- Inducing forced alkaline diuresis to increase salicylate excretion
- Exchange transfusion, hemodialysis, peritoneal dialysis, or hemoperfusion as needed in severe overdose
- Instituting blood transfusion and administering vitamin K_1 if necessary to treat haemorrhaging

Dosage and Administration

Adults:

80 to 325 mg to be taken every day, preferably at the same time each day. The tablets should be swallowed whole. Do not chew, break or crush the tablets as this will destroy the protective effect of the enteric coating.

Note : The information given here is limited. For further information, consult your doctor or pharmacist.
Storage : Store below 30°C.
Presentation/Packing : Blister pack of 10 x 10's and Strips of 10 x 10's.

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